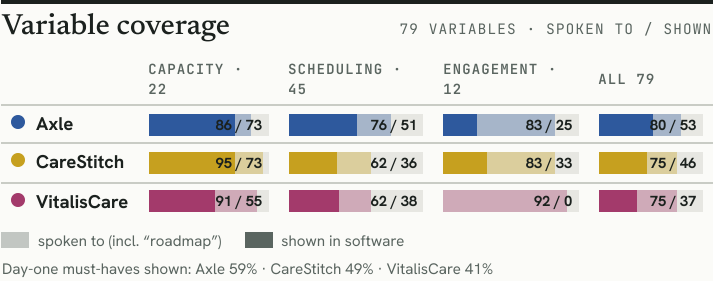


Three vendors, *one page*

Axle Health, CareStitch and VitalisCare scored on how much of our Variable Inventory each call covered, how far each product goes toward deciding for us, how much scheduler work each could remove, and what changes for the clinician.



Nobody covered 9 variables, 5 of them day-one must-haves: add-on orders, acuity to skill, cognitive/dementia constraint, diagnosis-driven timing, missed-visit documentation. Ask all three at the on-site.

The three things that decide valueOUR JUDGMENT FROM THE DEMOS · FIVE-POINT SCALE

	● AXLE	● CARESTITCH	● VITALISCARE
Product sophistication How far it goes from showing data to recommending and deciding	●●●●○ RECOMMENDS ALL THREE DECISIONS Ranks clinician, day and time with every factor shown; call-out cover suggested; zip-level forecast. Can run autonomously; customers keep a person in the loop.	●●○○○ FILTERS AND DISPLAYS Removes ineligible clinicians, then the scheduler judges. The request workflow orchestrates outreach well. Recommendations and AI are in research.	●●●○○ RECOMMENDS WHO AND WHEN Ranked provider matches and better-day suggestions with an AI rationale; overrides need a reason. Exceptions engine not shipped; no home health logic.
Scheduler task reduction How much of the 340 schedulers’ work it could take away	●●●●○ HIGH, IF THE DATA FLOWS Work queues to zero, bulk-out of the episode in one pass, a call-out in seconds. Claims ~74% scheduler time per visit, fully realized centrally. Needs HCHB faster than 3 h.	●●●○○ MEDIUM: COVERAGE, NOT ASSIGNMENT Kills the Teams and email coverage scramble and territory lookups. Every assignment is still a click. One customer doubled census with no new scheduler.	●●○○○ LOW TO MEDIUM Matching and day suggestions help, but plotting stays in HCHB, call-off recovery isn’t built, and a 3 h lag invites double entry in HCHB.
Clinician experience Perceived improvement for the clinician in the field	●●●●○ LESS EVENING PHONE TIME Optimize and finalize the night before; patients get a confirmation text instead of a call. Blocks with addresses. Opt-in rollout. The “why” goes to managers, not clinicians.	●●●●○ BUILT BY CLINICIANS Drive-time overlay, click-to-call with log, other disciplines’ visits, care-team chat, offline app, 93% adoption. But clinicians still phone every patient.	●●○○○ MIXED, WITH A TRUST RISK Routed day that reroutes live, but GPS workday tracking and route-adherence review, a second app beside PointCare, and no patient contact.

Other critical comparisonsFROM THE CALLS AND QUESTIONNAIRE RETURNS

	● AXLE	● CARESTITCH	● VITALISCARE
HCHB integration	Built, switched off. Reads the log ship and writes by RPA; needs data within ~10-15 min. Our intake feed covers new patients.	Read-only beta by screen automation; never in production; no write-back.	Live. Log ship in (~10 min typical), RPA writes out. Our 3 h feed is too slow for call-offs.
Home health logic	Native. Visit windows, Medicare week, front-loading, SOC, eval and recert queues. LUPA not shown.	None. “Guardrails live in the EMR.” No orders, auth or window enforcement.	Hospice-first. Auth, episode, LUPA and therapy all still to build.
Patient engagement	Live: two-way SMS, no patient app, replies routed to the clinician. Confirms rather than asks.	Human-led: click-to-call and call log. Text confirmation is on the top-five roadmap.	Nothing today; deprioritized for hospice. “Within scope.”
Footprint	~25 orgs · 80% home health · largest 11k ADC	79 orgs · 69% home health · largest 5.3k (one branch)	3 orgs live on Sync · ~6-7k patients · largest 3.5k ADC
Scorecard → suggested	63 Conditional → lift the hold, on-site with a joint data plan	56 Conditional → hold; follow-up on HCHB and automation R&D	67 Consider → hold; advance only on dated home health build terms

What each vendor must prove at the on-siteTHE QUESTION THAT DECIDES EACH ONE

● AXLE	● CARESTITCH	● VITALISCARE
- A joint data plan: intake feed for new patients, a target HCHB cadence for orders, and a date to turn write-back on. - The shared scenario run end to end, including LUPA pacing and a 10am referral decision. - Partnership terms, company size and a 5k+ ADC reference.	- A dated HCHB production plan, read and write. - The automation R&D they offered to show, and when recommendations ship. - A date for patient text confirmation, and a 60-day episode on the shared scenario.	- Written scope, dates and funding for the home health build: auth, episode and LUPA, therapy, engagement. - The Call-Off Center running live, including when nobody takes the visit. - Security and uptime (skipped on the call), size and funding.